

## Obesity Panel

Instructor: Janine Kyrillos, MD  
Block: Endocrine/ Urology/Reproduction  
Thread: Health Systems Science  
Conflict of Interest: Dr. Kyrillos is affiliated with the Novo Nordisk Speaker's Bureau and Advisory Board, Lilly Advisory Board

### Learning Objectives:

By exam time, you should be able to do the following:

- Identify obesity as chronic disease with multiple contributing factors
- Recognize treatment goals incorporating a weight inclusive approach
- Describe how bias and stigma affect people living with obesity
- Utilize person first and patient-centered language
- Interact with persons with obesity with empathy and compassion

### Study Questions:

1. Why should I address obesity as a disease?
2. How do my perceptions about weight impact my treatment of patients with obesity?
3. How can I talk to patients to help them make positive changes toward their health?

### I. What is Overweight and Obesity?

- Abnormal or excessive fat (energy) accumulation that may impair health
- Disruption in energy balance and homeostasis
- Chronic disease, requiring ongoing management, often with multiple modalities

### II. Prevalence of Obesity

- Estimated that 42% of US adults have obesity
- Over 2/3 of US adults have overweight or obesity

- Projection for 2030: >50% of US adults will have obesity
- Large variations by age, gender, race

### **III. Causes of Obesity**

- Highly impacted by social determinants of health
- Multiple complex biological, psychological, social, environmental, developmental, economic factors contribute to excess weight

### **IV. Evidence based treatment**

- Lifestyle and behavioral changes
- Weight-loss Medications
- Bariatric and metabolic surgery and procedures

### **VI. Attitudes and Bias**

1. Weight Normative Approach (Focuses unnecessarily on “normalizing” weight and BMI)
  - BMI Focused
  - Magic Numbers
  - Unrealistic Expectations
  - Learned Helplessness
  - Weight Cycling
  - Shame
  - Eating Disorders
2. Weight Inclusive Approach (Goal focused)
  - Health, decreased morbidity and risk
  - Energy
  - Confidence
  - Strength
  - Quality of life
  - Shared decision making

The Weight-Inclusive versus Weight-Normative Approach to Health: Evaluating the Evidence for Prioritizing Well-Being over Weight Loss  
<https://doi.org/10.1155/2014/983495>

### 3. **Attitudes of Healthcare providers** (physicians, physician assistants, nurses, dietitians, psychologists, fitness professionals, students in health-care fields)

- Non-compliant
- Lazy
- lack self-control
- awkward
- weak-willed
- sloppy
- unsuccessful
- unintelligent
- dishonest

Berryman et al., 2006; Brown, 2007; Ferrante et al., 2009; Hebl & Xu, 2001; Huizinga et al., 2009, 2010; Miller et al., 2013; Pantenburg et al., 2012; Phelan et al., 2014; Puhl et al., 2013, 2014; Swift et al., 2013; Vroman & Cote, 2011

### 4. **Actions of healthcare providers**

- Less time in appointments
- Less discussion with patients
- Less intervention
- Less respect to patients

Amy et al., 2006; Gudzone et al., 2013; 2014; Mensinger et al., 2018; Mulherin et al., 2013; Phelan et al., 2015; Turner et al., 2012

### 5. **Obesity stigma effect on access to care**

- Less likely to seek preventive care screening
- More likely to doctor shop
- 80% more likely to visit the emergency room... but no more likely to be hospitalized

### 6. **How Obesity Bias/Stigma Affects Behavior**

- 79% reported coping on multiple occasions by eating more food
- More frequent binge eating and refusal to diet
- More likely to avoid exercise
- If person feels judged, they are less likely to lose weight

- Puhl RM, Brownell KD. Confronting and coping with weight stigma: an investigation of overweight and obese adults. *Obesity* (Silver Spring) 2006;14(10):1802–1815
- Puhl RM, Moss-Racusin CA, Schwartz MB. Internalization of weight bias: implications for binge eating and emotional well-being. *Obesity* (Silver Spring) 2007;15(1):19–23
- Vartanian LR, Shaprow JG. Effects of weight stigma on exercise motivation and behavior: a preliminary investigation among college-aged females. *J Health Psychol* 2008;13(1):131–138
- Gudzune KA, Bennett WL, Cooper LA, Bleich SN. Perceived judgment about weight can negatively influence weight loss: a cross-sectional study of overweight and obese patients. *Prev Med*. 2014;62:103–107.

#### 7. Medical consequences to obesity bias and stigma

- Psychological
  - Anxiety, depression, low self-esteem, eating disorder, substance abuse, suicide
- Physiological
  - Increase in blood pressure, blood sugar, cortisol, C-reactive protein)
- Experiencing weight stigma predicts increased weight gain and obesity over time regardless of baseline BMI, age, race/ethnicity, and socioeconomic factors

Bucchianeri et al., 2014; Hatzenbuehler et al., 2009; Koball & Carels, 2011; Madowitz et al., 2012; Puhl, Himmelstein, Watson, 2019; Puhl & Luedicke, 2012, Sutin et al., 2018; Wu & Berry, 2017. Daly et al., 2019; Himmelstein et al., 2015; Major et al., 2012; McCleary-Gaddy et al., 2019; Sutin et al., 201; Schafer & Ferraro. 2011; Schvey et al., 2014; Sutin et al., 2014; Tomiyama et al., 2014; Tsenkova et al., 201

Wu & Berry, 2017 Puhl et al., 2017; Jackson et al., 2014; Schvey et al., 2019; Sutin & Terracciano, 2013; Sutin et al., 2014; Quick et al., 2013

## VII. Language in approaching patients with excess weight

### 1. People First Language

- Put the person before the disability or diagnosis.
- It is what the person *has*, not what the person *is*.
- Obesity is a disease. It doesn't define people
- Examples:

- ❖ A 45-year old, obese male with diabetes and colon cancer...
- ❖ A 45-year-old, cancerous male with obesity and diabetes...
- ❖ A 45-year-old male with obesity, diabetes, and colon cancer...

| Preferred Language               | Avoid            |
|----------------------------------|------------------|
| • Obesity                        | • Obese          |
| • Excess weight                  | • Morbidly obese |
| • Overweight                     | • Heavy          |
| • Weight issue or weight problem | • Fat            |
| • Higher weight                  |                  |
| • Body mass index                |                  |

## 2. Talk about weight objectively

- Tell me about your weight...
- Tell me about your child's weight...
- Is it ok if we talk about your weight for a moment?
- Let's look at your vital signs. Your BP is xx. I see your weight is up/down x pounds...

## 3. Non-Judgmental intervention

- Ask yourself if the patients weight relevant to their current issue? If it is, then consider linking
  - "Your weight may be putting extra pressure on your joints"
  - "Your weight may increase your risk for anesthesia"
- "We understand a lot more about weight. There are safe, effective treatment options. How can I help you get to the next step?"
- Approach obesity like any other diagnosis without judgement

## 4. Summary

- Approach all patients with empathy and respect
- You can make a dramatic impact on helping patients make meaningful changes
- Remember that people are doing the best they can with the tools they have

## Answers to Study Questions

*These answers are not meant to be all encompassing; rather they are examples of the types of thought processes needed for application of the material.*

### 1. Why should I approach obesity as a disease?

- Obesity has multiple complex etiologies, many of which are out of a patient's control
- Obesity and metabolic disease are linked to multiple metabolic, anatomic, and psychological diseases and conditions.
- There are effective, evidence-based treatment approaches to help patients improve their weight and metabolic health.

### 2. How do my perceptions of patients with obesity impact my treatment of patients with obesity in any medical field?

- There is known bias toward patients with excess weight and obesity, not only from physicians and health care providers, but from the general population. That bias has been shown to be associated with negative outcomes in patients including worsening of obesity and metabolic health and decrease in health screening and other access to care.
- This is primarily related to the experience of bias and feeling judged which interferes with building a trust relationship. Being aware of your biases may help you to compensate for them when you are treating obese patients.

### 3. How can I talk to patients to help them make positive changes toward their health

- Use person first language and speak to patients with compassion, dignity, and respect
- Discuss obesity and excess weight as a disease or medical condition with treatment options
- Address weight itself as just another data point and focus on broader health and wellness goals
- Remember that most people are doing the best they can with the tools they have.

## Resources

- Obesity Action Coalition <http://www.obesityaction.org/>
- The Obesity Medicine Association <https://obesitymedicine.org/>
- The Obesity Society <http://www.obesity.org/>
- Stop Obesity Alliance <https://stop.publichealth.gwu.edu/>
- UConn Rudd Center for Food Policy  
and Obesity  
<http://uconnruddcenter.org/weight-bias-stigma>
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